DISCHARGE SUMMARY

FICTIONAL SAMPLE DOCUMENT · FOR SOFTWARE DEMONSTRATION ONLY · NOT FOR CLINICAL USE

Patient	Anna Müller	**Discharge date**	24 October 2026
Age	62 years	**Clinician**	Dr. Weber (fictional)
Encounter	Inpatient discharge	**Follow-up**	3 November 2026, 10:30

1. Reason for admission and discharge

Fictional discharge diagnosis: Respiratory infection, improving. At discharge, the patient reports improved breathing and occasional cough with residual tiredness. This summary contains no actual examination findings, laboratory results, imaging results, or verified medical history.

2. Medication instructions

Take medicines only as listed on the separate, clinician-approved medication sheet. Do not independently change a dose or stop a prescribed medicine. **If a dose is missed, do not automatically take an extra or double dose.** Follow the instructions for that specific medicine and contact a pharmacist or healthcare professional if unclear.

3. Care at home

- Follow the rest and hydration guidance provided by the healthcare team.
- Gradually return to usual activities as tolerated, unless the written care instructions say otherwise.
- Keep this discharge summary and the separate medication sheet available for reference.

4. Warning signs and when to seek help

Seek emergency medical help immediately
Significantly worsening breathing, severe chest pain, confusion, or fainting.

Contact the healthcare team for advice
New or worsening symptoms that are concerning but do not appear to be an emergency. Do not wait for the scheduled appointment if advice is needed sooner.

5. Follow-up appointment

Date: Tuesday, 3 November 2026 **Time:** 10:30 a.m.
Please bring: the medication list and any questions or concerns.

6. Questions about medication or care

If instructions do not answer a question, contact the treating healthcare team or a pharmacist. Do not guess about medication changes. Confirm how to contact the healthcare team before leaving the facility.

DEMO SAFETY NOTICE
This fictional document was created to test PlainSpeak Lab's document parsing, evidence-grounded question answering, instruction extraction, and comprehension checks. It is not a real medical record and must not be used to guide patient care.